OCD-07 | For therapists | OCD, ages 8 and over

Support from others and independence

1 | Purpose, fit and boundaries

Purpose of this unit

With consent, map the difference between supporting the person and participating in an OCD cycle. Record roles defined by the therapist with the person and supporters.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Do not blame family, abruptly withdraw support or change responses without an individualized plan. Preserve warmth, connection, practical help and necessary protections. Sharing personal information requires appropriate consent and safeguarding obligations.

Reusing existing material

Unit 4 addresses the personal plan; unit 7 focuses on interaction and coordinated support. FAMILY supports discussion but does not replace professional decisions about participation in rituals.

A fictional example for discussion

Fictional example: Ido's family describes repeated reassurance requests before leaving home. The therapist separately hears Ido's wishes and the parents' difficulties, then explores a coordinated response. The sheet does not decide what to stop or when.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | A conversation about one situation

Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

1 | Question for discussion

Who is involved and whose help do I choose?

2 | Question for discussion

What help is necessary for functioning and what may participate in a ritual?

3 | Question for discussion

What was explained and agreed in treatment about supporters' responses?

4 | Question for discussion

How will we preserve relationships, privacy and practical needs?

5 | Question for discussion

What will we do with overload, disagreement or reassessment needs?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

Supporter disagreement or overload does not mean lack of care. First explore ability and boundaries. Without agreement or where harm is feared, avoid family response changes and consult a professional.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children aged 8–11 only

Ages 8–11 only. Read one question and allow brief or spoken answers. Parents add separate observations without deciding the child's thoughts; do not test reporting accuracy. Parents do not monitor the child's thoughts.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

Ages 12–17. Agree what stays private and what is shared with family or school, subject to safeguarding duties. Embarrassing thought content need not be disclosed. A parent agreement does not authorize blanket school sharing.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

Connect work to chosen goals, family, employment and existing care. Distinguish rituals, practical needs and value-consistent behavior. Participants may set limits on their ability to help.

OA | Older adults

Preserve adult language, choice and privacy. Assess access, health and desired support without automatically attributing new changes to OCD or assuming reduced ability. Medical or functional help is not stopped merely because it recurs.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

The P sheet proposes recording decisions and learning within treatment, not ERP instructions. The therapist determines suitability, action, scope and support. Without an agreed plan, use only for discussion. Do not begin exposure or change response prevention based on this sheet.

At the next session

Check whether coordination supported functioning, relationships and choice. Record misunderstood responses and return to the therapist before further change. Success is not measured by absence of requests or emotions.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Mapping against a full OCD manual and professional approval remain pending. This is original support material for review, not an approved ERP manual or its replacement.

www.nice.org.uk/guidance/cg31/chapter/Recommendations
iocdf.org/about-ocd/ocd-treatment-guide/erp/
kids.iocdf.org/professionals/md/treatment/

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